

Health Securitization and the 2024 Elections: The Past, Present and Future of Antimicrobial Resistance Global Governance

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Abstract

Antimicrobial resistance (AMR) is one of the most pressing global health and security threats of our time, killing 1.14 million people and annually costing the global economy USD 66 billion.^{1 2} This phenomenon, where diseases become resistant to the drugs used to treat them, is a natural evolutionary process that is wildly accelerated by the selective pressures caused by anthropogenic misuse of antimicrobials. The drivers are multisectoral and the problem necessitates strong coordination across the globe, pointing to the necessity of strong global health governance for this issue.³ AMR is relatively new to global governance, despite years of evolving under a securitization framework. However, the 2024 Election of Trump and the growing populist movement it represents threaten to reshape global governance and challenge the relatively new field of AMR global governance. This article analyzes the changes in global governance that the 2024 election posits, such as undercutting multilateralism, encouraging corporate interests, and challenging AMR governance in the process. To meet these threats, the WHO and UN must reform to strengthen the multilateral system, reinvigorate frameworks of operation and accountability mechanisms, and safeguard health outcomes through cross-cutting human rights reform, in order to safeguard against the threat of AMR.

¹ Naghavi et al., “Global Burden of Bacterial Antimicrobial Resistance 1990–2021.”

² “Forecasting the Fallout from AMR.”

³ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

The Global Problem of AMR

Antimicrobial resistance (AMR) is one of the most pressing global health and security threats of our time, directly killing 1.14 million people, being associated with 4.17 million deaths in 2021, and annually costing USD 66 billion to the global economy⁴. This phenomenon, where diseases become resistant to the drugs (including, but not limited to, antibiotics) used to treat them, is a natural evolutionary process that is wildly accelerated by the selective pressures caused by anthropogenic misuse of antimicrobials.

The drivers are multisectoral, and the problem necessitates strong coordination across the globe, pointing to the necessity of strong global health governance for this issue.⁵ AMR is relatively new to global governance, despite years of evolving under a securitization framework. Current scholars push for hard law governance to increase accountability.⁶ However, the 2024 Election of Trump and the growing populist movement it represents threaten to reshape global governance, and challenge the relatively new field of AMR global governance. Trump is likely to deprioritize AMR due to the economic counter costs,⁷ even though 2.8 million people in the US suffer from antibiotic-resistant infections annually, and more than 35,000 of that number die from it.⁸

This article analyzes the changes in global governance that the 2024 election posits, such as undercutting multilateralism, encouraging corporate interests, and challenging AMR governance in the process. To meet these threats, the WHO and UN must reform to strengthen the multilateral system, reinvigorate frameworks of operation and accountability mechanisms, and safeguard health outcomes through cross-cutting human rights reform, to safeguard against the threat of AMR.

Early Attempts at Regulation: Contain-and-Control (1907-1978)

AMR has been recognized for as long as antimicrobials have existed, with the first reports of “drug fastness” emerging in 1907. However, the appetite for regulation to address this issue was dampened by a persistent optimism that new drug discovery would outpace resistance.⁹ The few early regulatory attempts to address AMR through regulation were at the national level and have been piecemeal. As antibiotics became widely commercialized and used in the West during the 1930s and 40s, calls for “rational therapy” emphasized restrictions in antibiotic use for

⁴ Naghavi et al., “Global Burden of Bacterial Antimicrobial Resistance 1990–2021.”

⁵ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

⁶ Hoffman et al., “Governing the Global Antimicrobial Commons.”

⁷ Boucher, Murray, and Powderly, “Proposed U.S. Funding Cuts Threaten Progress on Antimicrobial Resistance.”

⁸ Amin et al., “2233. Burden of Bacterial Antimicrobial Resistance in United States in 2019.”

⁹ Podolsky, “The Evolving Response to Antibiotic Resistance (1945–2018),” 2.

select pathogens. During the 1950s, the outbreak of penicillin-resistant *Staphylococcus Aureus*, regulators relied on increased hand hygiene and updated antibiotic treatment regimens. This organismal control approach aimed to stop specific pathogens rather than limit the evolutionary pressures on microbial ecosystems, which created resistance.

The European Economic Community (EEC) recognized the multisectoral drivers of AMR by regulating therapeutic antibiotic use in agriculture in the 1970s, but their restrictions were limited, and parallel attempts in the US failed.¹⁰ While the adoption of AMR regulation by an international body such as the EEC elevated action beyond the national stage, this limited action and varied implementation highlight the early struggle in multilateral coordination of AMR regulation. Limited efforts from the global governance community failed to unite the global community on this issue: The technical aspects of laboratory standardization dominated WHO's activity on AMR from 1959 until the late 1970s, leaving AMR unaddressed in meaningful ways on the global stage.¹¹

One reason for the lack of global regulation was that infectious disease global governance (global ID) and the antimicrobial era were in their infancy at the same time. As the world globalized, European nations saw the value of international epidemiological surveillance and in standardizing infectious disease prevention and control regulations. They began collaborating through the International Sanitary Conventions (ISC) (1892-1926),¹² These conventions were explicitly developed to protect European security, trade and military interests from well-defined pathogens (ex, Cholera) crossing into their borders from Asia¹³, not to foster the conditions needed to prevent the emergence of pathogens within their own borders.¹⁴ As the global governance evolved, the ISC would become the International Health Regulations (IHR) we have today, but the foundational principle of health securitization persisted.¹⁵

Early national AMR regulation mirrored what Kirchelle and Podolsky call the “contain-and-control”¹⁶ consensus that the international community was simultaneously developing in ID governance: characterized by focusing on selective regulation and failing to address systemic drivers of resistance. However, AMR, driven by evolving pathogens within a state's borders, fell outside of this framework. Over subsequent decades, antimicrobial infrastructures— the systems and organization of AMR use, creation, and distribution in humans, livestock, agriculture, and more became embedded in daily life.¹⁷ These infrastructures were developed without active regulation at a global or national level, representing a missed

¹⁰ Kirchelle and Podolsky, 40-41.

¹¹ Podolsky, *The Antibiotic Era*, 150.

¹² Gostin and Meier, *Global Health Law and Policy*, chap. 6.

¹³ xenophobia

¹⁴ Kirchelle and Podolsky, “An Awkward Fit,” 41.

¹⁵ GOSTIN and KATZ, “The International Health Regulations,” 3.

¹⁶ Kirchelle and Podolsky, “An Awkward Fit,” 40.

¹⁷ Chandler, “Current Accounts of Antimicrobial Resistance.”

opportunity to ensure global coordination and multilateral consensus could develop for AMR, as antimicrobial infrastructure was first developed.

AMR enters the Global Stage (1978s-2000s)

Widespread attention to AMR began in earnest in the early 1980s, when NGO's spearheaded by physician and scientist activists sparked international efforts in controlling AMR and emphasized the one health, multisectoral drivers of this problem. Moreover, increasing reports of resistant pathogens such as MDR Tuberculosis, Vancomycin Resistant Enterococci, and MRSA by the end of the 90s fueled action. The WHO 1978 and 1982 conferences on AMR began their shift away from technical standards to managing governance frameworks. Indeed, a 1998 World Health Assembly resolution led to the 2001 *Global Strategy for Containment of Antimicrobial Resistance*, which provided a framework for state and non-state actors to address AMR.¹⁸ Concurrently, the Food and Agriculture Organization (FAO) and the Organization for Animal Health (OIE) shifted from reporting on AMR to proposing surveillance and stewardship frameworks,¹⁹ laying the foundation for a "One Health" approach which would further develop through joint reports and concept notes from 2003-2010^{20 21}

Many high-income nations and transnational organizations, such as the EU, began to develop their AMR policy. Despite increasing engagement, these responses mirrored the earlier national response's classic contain-and-control approach: selective regulations were imposed on use in animal and human health, especially in countries where antibiotic infrastructures characterized by widespread use in the agricultural and pharmaceutical industries provided an economic counter-interest to AMR regulations. One difference was the implementation of AMR surveillance by monitoring the evolution of resistance for specific, high-priority pathogens, but even this approach mirrored earlier regulatory effects in its selectivity and limited focus on specific pathogens, instead of electing to monitor broader indicators of resistance via background levels of AMR in the environment.²²

The similarities between early and later regulations are not just in the selectivity: the global health security paradigm was prominent in both. The ISC was explicitly created with the aim of ensuring global health security, and as it became the IHR, it would retain the security paradigm.^{23 24} However, limitations in scope (initially confined to 3 diseases and later unable to

¹⁸ Overton et al., "Waves of Attention"; Rabi, "The Global (Health) Governance of Antimicrobial Resistance."

¹⁹ Kirchhelle and Podolsky, "An Awkward Fit," 43.

²⁰ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

²¹ Overton et al., "Waves of Attention," 3.

²² Kirchhelle and Podolsky, "An Awkward Fit," 43.

²³ Kirchhelle and Podolsky, "An Awkward Fit," 43.

²⁴ GOSTIN and KATZ, "The International Health Regulations," 266.

be effectively implemented by the WHO) led to the dominance of other treaties in the fields of human rights, environmental protection, and trade in global health governance.^{25 26 27} Indeed, in the case of AMR, the IHR provides limited binding law but is not specific enough to AMR.²⁸ In the 21st century, the UN Security Council (UNSC) became a predominant player in infectious disease and pushed the framing of infectious disease as a threat to national security and the Global Health Security Agenda (GHSa) was formed from 44 countries and organizations as an extra-UN body to help the global community in its ability to prevent, detect, and respond to biological threats.^{29 30} In both the early regulations and newer global governance, AMR has been treated under the paradigm of global health security and eventually been characterized as a global security problem^{31 32}

The Current Era of AMR Governance

The GHSa was launched in 2014, and its first action paction was on AMR, creating three goals for states over the next five years: 1) developing National Action Plans (NAPs) 2) strengthening lab capacity and surveillance and 3) conserve existing treatments and develop new treatments (defined as antibiotics, preventative measures, and alternate treatments). They also created an Accountability and Results Task Force to support and guide AMR efforts, provide technical assistance, and foster accountability in implementing and maintaining NAPs.

Quickly thereafter in 2015, the tripartite adopted the nonbinding *Global Action Plan for AMR*, aiming to establish coordinated mechanisms to predict, detect and respond to AMR through five areas: 1) antimicrobial surveillance 2) infection prevention and control measures 3) global awareness campaigns 4) responsible use and stewardship programs and 5) innovation for new antimicrobials. It also aimed to promote sustainable development, ensuring prevention did not come at the cost of access and sustainability across sectors. It reaffirmed the need for NAPs to align with these goals, as well as whole-of-society engagement.

The 2016 High level Meeting (HLM) of the UN expanded awareness and provided engagement, resolving to support the development of the tripartite global action plan, committing to multisectoral collaboration under a One Health Approach, funding mobilization and raising awareness on AMR. It also called for the development of an Inter-Agency Coordination Group

²⁵ GOSTIN and KATZ, 265.

²⁶ Kirchhelle and Podolsky, "An Awkward Fit," 41.

²⁷ Gostin and Meier, *Global Health Law and Policy*, chap. 6.

²⁸ Kirchhelle and Podolsky, "An Awkward Fit."

²⁹ Sekalala, Williams, and Meier, "Global Health Governance through the UN Security Council," 27.

³⁰ GOSTIN and KATZ, "The International Health Regulations," 265.

³¹ Gostin and Meier, *Global Health Law and Policy*, chap. 6.

³² Chandler, "Current Accounts of Antimicrobial Resistance."

for AMR (IACG) to publish a report on future action of AMR, which occurred in 2019. This report maintained the consensus from the 2016 HLM.

Even more recently, in 2022 the United Nations Environmental Programme (UNEP) joined the tripartite and created the quadripartite, affirming the role of the environment in a One Health AMR approach.³³ They have continued to generate attention on AMR.

The culmination of global governance has been an HLM in 2024 calls for a reduction of AMR associated deaths by 10% relative to 2019 levels and for the creation of funded NAPs for 60% of countries by 2030 (while most countries have NAPs, only 11% set aside money for implementation as of 2023, and the raising of \$100 million USD. Stronger proposals were included in drafts, but gave way to these more general proposals and a lack of clear accountability mechanisms or funding frameworks.³⁴ These efforts mirror the concerns in previous soft law developments: global and national policies are underutilized and less effective due to a lack of multilateral cooperation and commitment to act.³⁵

These soft law frameworks have been criticized for accountability issues and for not taking bold action in the implementation of NAP's, so many scholars have called for a hard law instrument to redress this: a standalone or part of the new pandemic treaty.³⁶ However, the appetite for AMR action seems limited, as demonstrated by limited global development until recently and the lack of "appropriate urgency" described at the 2024 HLM.³⁷ The low efficacy of soft law instruments and the current lack of hard law treaties leaves the future directions of AMR uncertain.

Future Directions of AMR Governance Post-2024 US Election

The election of Donald Trump in the 2024 election marks a significant departure from America's longtime position as a leader in Global Health.³⁸ ³⁹Trump's previous actions suggest a shift from multilateralism to unilateralism, prioritizing American and corporate interests over global cooperation, with significant implications for global governance AMR governance. Indeed, the US used 6.5 % of the world's antimicrobials in 2017, making it the third highest consumer, so maintaining US involvement in AMR policy is vital.⁴⁰ The Trump administration's historical focus on corporate deregulation and previous undercutting of AMR global policy

³³ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

³⁴ Burki, "High-Level Meeting on Antimicrobial Resistance 2024."

³⁵ Gostin and Meier, *Global Health Law and Policy*, chap. 6.

³⁶ Gostin and Meier, chap. 15.

³⁷ Burki, "High-Level Meeting on Antimicrobial Resistance 2024."

³⁸ Noon, She, and Meier, "The US Elections as a Determinant of Global Health."

³⁹ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁴⁰ Tiseo et al., "Global Trends in Antimicrobial Use in Food Animals from 2017 to 2030."

implies the AMR policy will not be a priority.^{41 42} A lack of direct attention will not, however, stop the changes of the global health landscape from shaping the future of AMR governance.

Previous approaches to global health law and policy, which will have ramifications on AMR global governance in the future. AMR is a collective action problem because it requires the collective mobilization of many nations to be effective to preserve this common good, and the self-interest of any one nation can torpedo efforts globally.⁴³ However, the US has long been a leader in global policy forums, so its actions can change the landscape of the global governance field, thereby affecting AMR.⁴⁴ Trump's previous actions suggest a shift from multilateralism to unilateralism, prioritizing American and corporate interests over global cooperation, with significant implications for global governance AMR governance.

Departure from Multilateralism: Withdrawal from Global Health

The “America First” platform represents a turn to nationalism and isolationism.⁴⁵ Withdrawal from WHO, Paris Climate Agreement, and threats to GHSA and PEPFAR in his previous term indicate a retreat from global health governance is likely in the second term (Noon et al.). Trump alleges Chinese control of the WHO⁴⁶ and the need for WHO reform as his rationale for withdrawal from this organization.⁴⁷ His rationale for withdrawal from the Paris Climate agreement involves the argument that this deal limits US industrial power unfairly and empowers other nations. This corporate interest argument again alleges Chinese influence in attempting to limit US industrial power through the accords. Some argue this may constitute a negotiation tactic to further US positions in these treaties,⁴⁸ but as it stands, the US remains withdrawn from many aspects of global governance.

Departure from Multilateralism: Endorsing Unilateral Goals in Global Health

Where the US remains in global governance, it is likely to push for unilateral goals along the lines of increasing its position in global governance, corporate priorities, and ideological interests. Withdrawal from the WHO and climate accords, with an assumed willingness to reenter under more “favorable terms” for US industrial power and through the limiting of Chinese power on the global stage demonstrates an intention to increase corporate power and

⁴¹ Jacobs, “Opposition to Breast-Feeding Resolution by U.S. Stuns World Health Officials.”

⁴² Boucher, Murray, and Powderly, “Proposed U.S. Funding Cuts Threaten Progress on Antimicrobial Resistance.”

⁴³ Gostin and Meier, *Global Health Law and Policy*, chap. 6.

⁴⁴ Noon, She, and Meier, “The US Elections as a Determinant of Global Health.”

⁴⁵ Noon, She, and Meier.

⁴⁶ Xenophobia, again.

⁴⁷ Callahan, “Inside the Trump Administration’s Decision to Leave the World Health Organization.”

⁴⁸ Zhang et al., “The Withdrawal of the U.S. from the Paris Agreement and Its Impact on Global Climate Change Governance.”

limit enemies through global governance and especially the global health playing field.^{49 50} Moreover, his reinstatement of the global gag rule prevents US foreign aid recipients' engagement in abortion-related activities.⁵¹ Mexico City policy further prohibits US foreign aid recipients from engaging in abortion-related activities.⁵² This unilateral focus is also concerning because of the US's influence in other areas of global governance. For example, the first Trump admin opposed the Breastfeeding Substitutes Marketing Resolution, which results in harmful formula marketing in areas where it is unsafe to use due to unclean water. The US failed to move consensus on the treaty, so they aimed to torpedo the resolution by threatening the resolution's sponsor, Ecuador, with military aid withdrawal and trade sanctions. LMIC from Africa and Latin America feared stepping in, and ultimately, the resolution was only saved because Russia sponsored it.⁵³

Though AMR is a problem which affects all nations— indeed, 2.8 million people in the US suffer from antibiotic-resistant infections annually, and more than 35,000 of that number die from it,⁵⁴ but the burden is disproportionately borne by LMIC countries, and framing has shifted towards the global north protecting the global south.⁵⁵ With counter-economic interests, the US is likely to upend multilateralism for AMR the same way it did so with the Breastfeeding Substitutes treaty to promote economic and corporate interests. AMR has less established consensus and delegates are less urgent to address it, casting uncertainty on who, if anyone, would step in to counter the US. However, shifting away from AMR governance, framed around the global south, will not save the global north and America from the life and economic costs AMR will incur. Previously, discordant attempts at national regulation show that AMR cannot be solved without strong multilateral cooperation. AMR, by its nature, is a collective action problem, which requires the collective mobilization of many nations to be effective in preserving this common good of antimicrobials. The self-interest and unilateralism of one nation can crush global stewardship efforts for AMR, and without strong global governance, no nation will escape this threat.⁵⁶

While using the global health stage to endorse unilateral interests is not new or solely done by the US or the Trump administration, the expansion of its scope to unprecedented actions raises concern and threatens to severely shift global governance and the balance of global power.

Global Resource Dynamics

⁴⁹ Callahan, "Inside the Trump Administration's Decision to Leave the World Health Organization."

⁵⁰ Zhang et al., "The Withdrawal of the U.S. from the Paris Agreement and Its Impact on Global Climate Change Governance."

⁵¹ Gostin and Meier, *Global Health Law and Policy*, chap. 17.

⁵² Noon, She, and Meier, "The US Elections as a Determinant of Global Health."

⁵³ Jacobs, "Opposition to Breast-Feeding Resolution by U.S. Stuns World Health Officials."

⁵⁴ Amin et al., "2233. Burden of Bacterial Antimicrobial Resistance in United States in 2019."

⁵⁵ Overton et al., "Waves of Attention."

⁵⁶ Gostin and Meier, *Global Health Law and Policy*, chap. 6.

The withdrawal from the WHO constitutes a loss of one-fifth of WHO's budget, eviscerating its capacity to address AMR as part of the quadripartite (WHO, FAO, OIE, UNEP).⁵⁷ Moreover, The Institute for Health Metrics and Evaluation estimates that the U.S. was responsible for \$20 billion of governments' development assistance in 2023, constituting over 40% of this funding.⁵⁸ In the realm of infectious disease, it is likely to withdraw from the Global Fund for AIDS, Tuberculosis, which also provides grants and supports health systems strengthening with its budget.⁵⁹ ⁶⁰ These funding cuts will limit the scope of Global Health Governance, limiting action on policies like AMR, which already suffer from a lack of delegate urgency. Moreover, building equitable underlying health systems is vital in preventing AMR, so cutting this funding will have long-term impacts on nations' abilities to respond to AMR.⁶¹

What funding remains from the US will likely be earmarked for specific goals the US wishes to achieve, due to the evidence of increased endorsement of unilateral goals by the Trump administration. This will increase a growing trend of bilateral funding dominating the global health landscape, which creates chronically overfunded and underfunded health goals.⁶² Bilateral funding may be combined with economic sanctions to influence specific nations' health systems and involvement in governance in ways that are not in their best interest, as demonstrated by the case of the Breastfeeding Substitutes Resolution.⁶³

Moreover, the US withdrawal from the Paris Agreement marked a shift from a global supplier to a consumer of support resources for climate change.⁶⁴ A similar shift was noted in infectious disease, with the US stockpiling vaccines instead of distributing them, and purchasing of the entire global supply of remdesivir.⁶⁵ ⁶⁶ Potential US funding cuts to GAVI will exacerbate these stockpiling behaviors, which limit vaccination resources for other nations.⁶⁷ This shift to consumer behavior by the US in key global health areas will strain limited resources alongside withdrawn funding, threatening health equity across nations. In the case of antimicrobial resistance, funding is vital in taking NAPs from ideas to reality, with the WHO calling for USD\$100 million to support their goals at the previous HLM.⁶⁸ Moreover, if the US follows their consumer behavior, they are likely to stockpile material resources such as antimicrobials. This will exacerbate global supply issues and increase the inequitable distribution of key resources in combating AMR.

⁵⁷ "What Is the World Health Organization Without the United States?"

⁵⁸ Patnaik, "Global Health Insecurity in a Second Trump Term [Guest Essay]."

⁵⁹ "The Global AMR Innovation Fund."

⁶⁰ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁶¹ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

⁶² Gostin and Meier, chap. 5.

⁶³ Jacobs, "Opposition to Breast-Feeding Resolution by U.S. Stuns World Health Officials."

⁶⁴ Zhang et al., "The Withdrawal of the U.S. from the Paris Agreement and Its Impact on Global Climate Change Governance."

⁶⁵ Bollyky and Bown, "Vaccine Nationalism Will Prolong the Pandemic."

⁶⁶ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁶⁷ Gostin, Moon, and Meier.

⁶⁸ Burki, "High-Level Meeting on Antimicrobial Resistance 2024."

Foundations such as the Bill and Melinda Gates Foundation have previously stepped in to fill state funding gaps, but this poses an issue of accountability, as this NGO is unaccountable to human rights.⁶⁹ Major funders such as Germany or the UK could also increase funding, but other political and global pressures may pull attention and funding away from global health.⁷⁰

Global Power Dynamics

In an age of populism, Trump's unilateral tactics on the global stage may enable other populist nations to turn further inward. Negotiation theorists highlight how, as the US unilaterally withdraws from global health governance institutions, it changes "the rules of the game" or processes of global governance. While this can be viewed as a success in the short term, starting negotiations, the heavy reliance on unilateral forcing risks tit-for-tat unilateral responses and superficial compliance.⁷¹ This may spark an increase in unilateral tactics used by other nations, undermining multilateralism, especially due to an increasing turn of populism.⁷² Trump's unlawful withdrawal from the WHO⁷³ and violations of the IHR⁷⁴ may result in increased disregard of hard law and binding policies. Similarly, Trump affirmed the need for collective action in AMR with the G20, but quickly undercut this through economic policy with public health congressional cuts to global surveillance, prevention, laboratory capacity, and coordination.⁷⁵⁷⁶ Increased unilateralism and disregard for established norms of governance have the potential to change the face of global health governance.⁷⁷

These actions may also alter global power dynamics. Indeed, in light of US withdrawal from leadership, China has expanded its influence and alliances.⁷⁸ Ironically, upon the US withdrawing from the Trans-Pacific Partnership (TPP)- China increased negotiations with the remaining parties, strengthening its power over this agreement.⁷⁹ Withdrawal from global health governance has led China to expand its development agenda and, therefore, its soft power agenda

⁶⁹ Gostin, Constantin, and Meier, "Global Health and Human Rights in the Age of Populism," chap. 20.

⁷⁰ Sullivan, "COMMENT."

⁷¹ "The Risks and Benefits of Unilaterally Changing the Rules of the Game - ProQuest."

⁷² Gostin, Constantin, and Meier, "Global Health and Human Rights in the Age of Populism," chap. 20.

⁷³ "US Withdrawal from WHO Is Unlawful and Threatens Global and US Health and Security - The Lancet."

⁷⁴ Habibi et al., "Do Not Violate the International Health Regulations during the COVID-19 Outbreak."

⁷⁵ Boucher, Murray, and Powderly, "Proposed U.S. Funding Cuts Threaten Progress on Antimicrobial Resistance."

⁷⁶ "G20 Leaders Make Historic Commitment to Combat AMR and TB in Declaration."

⁷⁷ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁷⁸ Gostin, Moon, and Meier.

⁷⁹ "The Risks and Benefits of Unilaterally Changing the Rules of the Game - ProQuest."

in sub-Saharan Africa.⁸⁰ The power structure between major powers such as the US, EU, and China has invariably changed due to the US's withdrawal from multilateralism.⁸¹

Trump's actions in the Paris Climate accord also rebuke the principles of Common but Distributed Responsibility, or CBDR, which imposes higher mitigations on already developed countries that have benefited from pollution and have disproportionately polluted the world. His attack on this principle would see developing countries, which both have not benefited from industrial technology as much and bear the disproportionate impacts of climate change, raise their mitigation goals and provide more support to other countries.⁸² This withdrawal from the consensus of CBDR raises an equity issue across nations in global health proceedings. AMR is similarly a problem that is disproportionately borne by developing countries⁸³ and where just action requires the support by developed countries of developing countries, not the other way around, and a shift in these attitudes may undermine equity and progress in AMR governance.

Policy Implications

The 2024 Elections are likely to change the face of global health governance, due to Trump's shifts away from multilateralism and towards corporate and unilateral interests, and the large implications of Trump's US on other nations and the norms of global governance. As the world turns towards populism and isolationism, the global governance system faces a choice: reform to strengthen the multilateral system, reinvigorate frameworks of operation, and safeguard health outcomes through cross-cutting human rights reform, or dwindle in power. The fate of the world's most pressing health challenges, including antimicrobial resistance, hangs in the balance. Trump is unlikely to engage directly with AMR due to corporate counterinterests, and the appetite for a hard law treaty is low, so in the near future, the soft law consensus is the most likely path forward. However, work can be done to shore up accountability and ensure progress.

Resilient Multilateralism

⁸⁰ Sullivan, "COMMENT."

⁸¹ Zhang et al., "The Withdrawal of the U.S. from the Paris Agreement and Its Impact on Global Climate Change Governance."

⁸² Zhang et al.

⁸³ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

A resilient landscape, which acknowledges the populist nationalism among member states but encourages coordination, must be developed.⁸⁴ As the world changes, both with the election of Trump and the rise of populism, governance systems risk falling behind and being unable to accommodate new governance. Thus, significant reforms must allow for changing ways in how countries, especially dominant powers, interact with each other. This can limit withdrawal while still protecting the rights of other nations and a fair system.

In upholding multilateralism, reform in financing mechanisms must be considered. This is because bilateral funding threatens to warp priorities and fails to protect nations from others who would impose, giving a tool to achieve unilateral objectives. Mandatory funding with which can be evenly distributed across a wide range of health issues, not just those donor states prioritize, will ensure effectiveness and equity in funding allocations.⁸⁵ Moreover, though Director-General Chan's proposal to increase mandatory contributions was unsuccessful in late 2016,⁸⁶ the surge of activity around the pandemic treaty and the memory of the recent catastrophe as a call for more funding.

Moreover, as funding is inevitably slashed, effort must be made to resist funneling remaining resources into narrow, vertical initiatives. Though narrow fixes are cheaper than the systematic restructuring of health systems, the WHO will always be operating under a budget deficit, and cannot let this distract from the horizontal solutions that will help them reach their broad mandate of "a state of complete physical, mental, and social well being and not merely the absence of disease or infirmity" for all.⁸⁷ Again, the memory of COVID and the necessity of broad solutions and wide structural changes that became apparent should be used to motivate these financial changes.⁸⁸ Indeed, achieving that mandate requires recognition of the wide structural changes in creating health, especially in the case of AMR, where limited technical solutions are eventually doomed to become ineffective.

Involving the US in AMR Policy: Global Health Security vs Human Rights

As discussed, Infectious disease and AMR operate under a paradigm of global security and traditional "contain-and-control" approaches to managing pathogens. The securitization of AMR and other infectious disease issues bode well for US involvement,⁸⁹ as after Trump announced a withdrawal from WHO, US agencies were instructed that they could only work with WHO "on the grounds of national security and public health safety."⁹⁰ This posits a

⁸⁴ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁸⁵ Gostin and Meier, *Global Health Law and Policy*, chap. 5.

⁸⁶ Gostin and Friedman, "Global Health."

⁸⁷ Lane, "Good Health Is More than Just the Absence of Illness » NCRC."

⁸⁸ Gostin and Meier, *Global Health Law and Policy*.

⁸⁹ Youde, "The Securitization of Health in the Trump Era."

⁹⁰ Callahan, "Inside the Trump Administration's Decision to Leave the World Health Organization."

framework for keeping the US engaged in global governance, either through hard law or soft law, a key part of attaining multilateralism in AMR governance.

However, engaging with public health only on the grounds of securitization risks limits the human rights considerations of health and pushes the responsibilities onto LMIC's, creating an inequitable landscape of governance and poor health outcomes.⁹¹ A biosecurity paradigm could risk worsening AMR outcomes, especially because AMR disproportionately impacts LMICs, and nationalist policies both worsen inequities and violate human rights.⁹² Indeed, health securitization often fails to accurately represent countries responses: for example, the US and the UK were ranked #1 and #2 on the Global Health Security Index (GHSI), meant to quantify nations' ability to respond to infectious disease threats, but performed poorly in terms of controlling the spread of disease compared to countries in East and SouthEast Asia, which had lower GHS rankings.⁹³ However, securitization is still a valuable framework that can often compel important healthcare protections while motivating actors. Indeed, the GHSA focuses on ensuring common access in AMR.⁹⁵ It is worth noting that Trump has attacked the GHSA and may withdraw funding⁹⁶, threatening the equity already incorporated into biosecurity frameworks. While securitization may prove a valuable rhetorical framework for engaging populist nations such as the US, this paradigm must be interrogated, and human rights must be fully integrated into it. This requires the integration of principles of equity, social justice, and human rights to provide moral foundations in global health and shape future norms. These policies are the antithesis of nationalism and provide a basis for collaborative governance, or cosmopolitanism, in global health; thus, they must be instilled as cross-cutting frameworks throughout the WHO and UN system to ensure health outcomes are protected and equitable in the case of AMR.⁹⁷

Increasing the Appetite for AMR Policy

⁹¹ Overton et al., "Waves of Attention"; Sekalala, Williams, and Meier, "Global Health Governance through the UN Security Council"; Mahajan, "Casualties of Preparedness."

⁹² Habibi et al., "Do Not Violate the International Health Regulations during the COVID-19 Outbreak"; Gostin and Meier, *Global Health Law and Policy*, chap. 15. Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁹³ "The Global Health Security Index and Its Role in Shaping National COVID-19 Response Capacities: A Scoping Review - PMC."

⁹⁴ Mahajan, "Casualties of Preparedness."

⁹⁵ Gostin and Meier, *Global Health Law and Policy*, chap. 15.

⁹⁶ Gostin, Moon, and Meier, "Reimagining Global Health Governance in the Age of COVID-19."

⁹⁷ Sekalala, Williams, and Meier, "Global Health Governance through the UN Security Council"; Gostin and Meier, *Global Health Law and Policy*, chap. 4; Gostin, Constantin, and Meier, "Global Health and Human Rights in the Age of Populism," chap. 20.

Trump is unlikely to engage directly with AMR due to corporate counter interests ⁹⁸and the appetite for a hard law treaty is low,⁹⁹ so in the near future, the soft law consensus is the most likely path forward. The major criticism of current soft law is its lack of accountability frameworks. However, work can be done to shore up accountability and ensure progress within the soft law domain. Indeed, hard and soft law often have similar accountability mechanisms, and when policies are newer and consensus has not been reached, soft law can be a powerful tool in generating activity and slowly developing consensus. ¹⁰⁰ One important mechanism of accountability that can be used in soft law is “naming and shaming,” essentially using civil sector pressure to hold officials accountable. Most civil sector action focuses on changing behavior in antibiotic consumption, so there is little awareness of the structural drivers of AMR and the need for global governance. ¹⁰¹ Indeed, the 2024 HLM included representatives who lacked “urgency,” highlighting the need for engagement from the civil sector in pressuring their governments to revitalize AMR global action, adding accountability to the development of AMR through soft law policy. ¹⁰²

Conclusion

Antimicrobial resistance (AMR) remains one of the most urgent global health challenges, with an immense amount of human life and global security at risk. The natural evolutionary process of resistance, exacerbated by anthropogenic misuse of antimicrobials, makes it a global problem, requiring robust, multilateral, and multisectoral coordination and strong global health governance. The 2024 U.S. election and the populist movement it represents threaten to deprioritize AMR governance and change the shape of global governance, undermining multilateralism and favoring corporate interests over public health. These dynamics pose significant risks to the progress made in this nascent field. To mount a strong response, the WHO and UN must reform to fortify multilateral systems, consider the limitations of their frameworks, and integrate human rights reforms to protect health outcomes worldwide. This reformed system, combined with an effort to increase accountability methods for the civil sector, will be necessary in future AMR governance. Only through decisive action can the global community effectively combat the rising tide of AMR and its far-reaching consequences.

⁹⁸ Jacobs, “Opposition to Breast-Feeding Resolution by U.S. Stuns World Health Officials.”

⁹⁹ Burki, “High-Level Meeting on Antimicrobial Resistance 2024.”

¹⁰⁰ Sekalala, Williams, and Meier, “Global Health Governance through the UN Security Council.”

¹⁰¹ Chandler, “Current Accounts of Antimicrobial Resistance.”

¹⁰² Burki, “High-Level Meeting on Antimicrobial Resistance 2024.”

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